National Cancer Control Programme (NCCP) 00309 · clinician draft

Weekly carboplatin + paclitaxel with radiotherapy

> **Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Week 1 · antiemetic plus paclitaxel reaction prevention

Active medicines: Paclitaxel + Carboplatin

Emetogenic risk: Moderate

Medicine	Dose / route	When
Ondansetron	16 mg orally	Before weekly chemotherapy Review QT/constipation risk.
Dexamethasone antiemetic reference	8 mg orally Day 1 and Days 2–3	NCIS moderate-risk course Reconcile with paclitaxel steroid premedication; one approved schedule.
Metoclopramide	10 mg orally three times daily as needed	Subsequent days Check contraindications and duration.
Dexamethasone paclitaxel reference	8 mg intravenous (IV)	30 minutes before paclitaxel National Cancer Control Programme (NCCP) reaction-prevention dose; do not duplicate the antiemetic steroid.
Chlorphenamine	10 mg IV	30 minutes before paclitaxel
Famotidine	20 mg IV	30 minutes before paclitaxel Consultant/local-policy decision on omission.

Additional phase-specific support

- Reconcile paclitaxel premedication and antiemetic dexamethasone into ONE prescription; do not add duplicate steroids.
- NCCP allows consultant-led famotidine omission without prior hypersensitivity, while noting uncertain reaction risk; close monitoring and reaction facilities remain necessary.
- Muscle/joint pain: consider prescribed analgesia. Assess swallowing, nutrition and respiratory symptoms during and after radiotherapy (RT).
- No routine granulocyte colony-stimulating factor (white-cell growth support) (G-CSF), antibiotic, antiviral or Pneumocystis jirovecii pneumonia (PJP) prophylaxis is mandated in this source; assess individual risk/local policy.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00309 v6a · pNCCPpp4–5;NCISp4 · NCCP NCIS medical-oncology antiemetics v8

Sources checked 2026-11-02 · SACTCheck v0.84.30 · Regimen-level drafts; prescriber and oncology-pharmacy verification required

Day 8 onward · review prior reactions each week

Active medicines: Paclitaxel + Carboplatin

Emetogenic risk: Moderate

Medicine	Dose / route	When
Ondansetron	16 mg orally	Before weekly chemotherapy Review QT/constipation risk.
Dexamethasone antiemetic reference	8 mg orally Day 1 and Days 2–3	NCIS moderate-risk course Reconcile with paclitaxel steroid premedication; one approved schedule.
Metoclopramide	10 mg orally three times daily as needed	Subsequent days Check contraindications and duration.
Chlorphenamine	10 mg intravenous (IV)	30 minutes before paclitaxel
Famotidine	20 mg IV	30 minutes before paclitaxel May be omitted if no hypersensitivity under consultant guidance.

Medicine	Dose / route	When
Paclitaxel-specific dexamethasone	None routinely in this source table from Day 8	May be added for increased risk or a prior reaction This does not cancel the separate carboplatin antiemetic steroid. Consultant may prescribe20 mg orally12 hours and6 hours before a carefully authorised rechallenge; severe paclitaxel reactions require separate specialist management and no routine rechallenge.

Additional phase-specific support

- Reconcile paclitaxel premedication and antiemetic dexamethasone into ONE prescription; do not add duplicate steroids.
- National Cancer Control Programme (NCCP) allows consultant-led famotidine omission without prior hypersensitivity, while noting uncertain reaction risk; close monitoring and reaction facilities remain necessary.
- Muscle/joint pain: consider prescribed analgesia. Assess swallowing, nutrition and respiratory symptoms during and after radiotherapy (RT).
- No routine granulocyte colony-stimulating factor (white-cell growth support) (G-CSF), antibiotic, antiviral or Pneumocystis jirovecii pneumonia (PJP) prophylaxis is mandated in this source; assess individual risk/local policy.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____